

AETHERA · SPECIALTY BILLING SERIES

Gastroenterology Billing Solutions

Maximizing Revenue Across Office E/M, Endoscopy, Hepatology, IBD Biologics,
and Bowel Prep

Prepared by: Aethera Healthcare Solutions · Lakeland, FL

Scope: CPT 43200-45399 · 91110-91122 · 99202-99215 · J-codes for IBD biologics

Updated: 2026 Edition · aetherahealthcare.com · +1 (863) 694-0325





ABOUT AETHERA

Maximizing Revenue. Minimizing Burden.

Aethera Healthcare Solutions is your full-service medical billing partner headquartered in Lakeland, FL. We handle coding, claims, payments, denials, and collections — so you can focus on patients.

500+

Practices Served

Across 25+ medical specialties

\$12M+

Revenue Recovered

Denied & underbilled claims recouped

95%

Clean Claim Rate

vs 84% industry avg

< 30d

Average AR

Days in accounts receivable



Personal, Not Corporate

Dedicated account team with deep specialty/payer knowledge.



Performance Targets, In Writing

96%+ net collection, <5% denial, 95%+ coding accuracy, 24h posting, 48h submission, 65%+ appeal success.



Zero Risk to Start

No setup fees, no long-term contracts, 90-day cancel, free 5-day audit.



GASTROENTEROLOGY • MARKET CONTEXT

Why Gastroenterology Billing Matters Now

Gastroenterology is one of the highest-volume procedural specialties — 19M+ colonoscopies annually in the US. The screening-vs-diagnostic distinction determines whether Medicare pays \$0 (screening, fully covered) or \$1,200+ (diagnostic, with deductible). IBD biologic spend exceeds \$8B/year.

\$92B

**US digestive disease treatment spend
(2024)**

Largest single specialty spend after
cardiology.

19M+

Annual colonoscopies performed in US

15M screening, 4M diagnostic;
Medicare covers screening every 10 yrs.

~ 16%

GI claim denial rate (industry avg)

Driven by screening-vs-diagnostic
miscoding and modifier issues.

\$8B+

**Annual US IBD biologic spend (Crohn's,
UC)**

Remicade, Humira, Stelara, Entyvio
dominate.



Net effect: GI practices that master screening-vs-diagnostic colonoscopy coding, IBD biologic PA, and endoscopy modifier -51/-59 use can recover **7–12%** of total revenue currently lost to denials and under-billing.



GASTROENTEROLOGY · OVERVIEW

The Four Pillars of GI Billing

Each pillar represents a distinct service family with unique coding, modifier, and payer requirements.

◆ 01 · OFFICE E/M

Office E/M & GI Consultations

Office visits (99202–99215) for GI complaints, IBD management, liver disease. Telehealth allowed with modifier 95.

CPT 99202–99215 · Modifier 25 w/ procedures



02 · ENDOSCOPY

Endoscopy Procedures

Colonoscopy (45378–45398), EGD (43200–43259), ERCP (43260–43276), flexible sigmoidoscopy (45330–45346). Screening vs diagnostic distinction drives coverage.

CPT 43200–45399 · Screening vs diagnostic

◆ 03 · HEPATOLOGY

Hepatology & Liver Disease

Hepatitis C antivirals (Mavyret, Eplusa — \$24K–\$96K per treatment course), liver biopsy (47000–47001), hepatocellular carcinoma management.

J-codes HCV · 47000–47001 · PA mandatory



+ 04 · IBD BIOLOGICS

IBD Biologic Infusions

Remicade (J1745), Humira (J1745), Stelara (J3357), Entyvio (J3590). Office infusion (96365) vs hospital infusion — site-of-service rules.

J1745 · J3357 · 96365 · Site-of-service



-pillar 01 · OFFICE E/M

Office Visits & GI Consultations

GI office encounters cover IBD management, liver disease follow-up, reflux, abdominal pain, and pre-procedure evaluations. The 2021 E/M coding reform shifted documentation to MDM-based. Modifier 25 is required when billing E/M + procedure (injection, infusion) on the same day. Pre-procedure H&P for screening colonoscopy is now bundled — cannot bill separately.



E/M coding (2021+ reform)

Code by Medical Decision Making complexity or time; 99202–99215. Modifier 25 required if same-day procedure.



Pre-procedure H&P

Pre-colonoscopy H&P is bundled into colonoscopy code; do NOT bill separate E/M. Exception: medical condition requiring separate evaluation (e.g., new cardiac symptom).



Telehealth GI visits

Allowed for follow-up IBD, medication management, post-procedure check. Modifier 95 + POS 02. Parity required in 42+ states.



Modifier 25 misuse is the #1 GI audit trigger — appending to E/M when procedure on same day does not represent significant, separately identifiable service.



Top GI E/M CPT Codes

Code	Service	Avg \$
99213	Est. E/M, low MDM	\$92
99214	Est. E/M, moderate MDM	\$131
99215	Est. E/M, high MDM	\$187
99203	New patient, low MDM	\$113
99204	New patient, moderate MDM	\$167

Medicare national avg professional component, 2026 edition. MDM = Medical Decision Making.



PTLLAR 02 · ENDOSCOPY

Colonoscopy, EGD, ERCP & Screening Coding

Endoscopy is the procedural backbone of GI. The single most important coding distinction is screening vs diagnostic colonoscopy — same procedure, dramatically different payer coverage. Screening colonoscopy (45378 with PT modifier) is fully covered under ACA preventive; diagnostic colonoscopy (45378 with diagnostic diagnosis) applies deductible.

◆ Colonoscopy

45378 diagnostic (\$750); 45380 with biopsy (\$950); 45384 with polyp removal hot biopsy (\$1,100); 45385 with snare polypectomy (\$1,250). Screening: G0121 (colorectal screening, average risk) or G0105 (high risk).

ⓘ EGD (Upper Endoscopy)

43200 diagnostic EGD (\$575); 43202 with biopsy (\$725); 43239 with foreign body removal (\$825); 43244 with band ligation (\$1,150).

↔ Screening vs Diagnostic

Screening colonoscopy: G0121 (avg risk, 10yr), G0105 (high risk, 5yr) — covered 100% under ACA. Diagnostic 45378 with symptom/disease diagnosis: applies deductible + coinsurance.

⚡ Modifier 33 (Preventive)

Append modifier 33 to screening colonoscopy to indicate preventive service; bypasses deductible. When polyp found and removed, screening converts to diagnostic — patient cost-share may apply.

🕒 ERCP

43260 diagnostic ERCP (\$1,400); 43264 with stone removal (\$1,800); 43271 with stent placement (\$2,100). High-complexity procedural code.

◆ Flexible Sigmoidoscopy

45330 diagnostic (\$325); 45333 with biopsy (\$425); 45339 with polyp removal (\$550). 5-year screening interval for average risk.

◆ Polyp Found — Conversion

If screening colonoscopy finds and removes polyp, code 45380–45385 (with PT modifier) — converted to diagnostic. Patient cost-share may apply; inform patient in advance.



-pillar 03 · HEPATOLOGY

Hepatitis C Antivirals & Liver Disease Management

Hepatology is one of the highest-cost GI sub-specialties due to hepatitis C antiviral drug costs. A 12-week course of Mavyret (glecaprevir/pibrentasvir) costs \$24,000–\$39,000; Epclusa (sofosbuvir/velpatasvir) costs \$74,000–\$96,000. Pre-auth mandatory; documentation of HCV genotype, fibrosis stage, and treatment-naïve vs experienced status required.

HCV antivirals

Mavyret \$24K–\$39K (8–12 weeks); Epclusa \$74K–\$96K (12 weeks); Harvoni \$94K (12 weeks); Zepatier \$54K (12 weeks). Cure rates >95%.



Liver biopsy

47000 percutaneous liver biopsy (\$425); 47001 with imaging guidance (\$575). Often required to stage fibrosis before HCV treatment.



HCC management

Hepatocellular carcinoma: TACE (transarterial chemoembolization, 37243), radiofrequency ablation (47382), Y-90 (37243). High-cost procedures with strict PA.



HCV antiviral pre-auth is the highest-value PA workflow in hepatology — one denied Mavyret claim = \$24K+ revenue loss. Step therapy (interferon-based) is now obsolete; modern DAAs are first-line.

HCV antiviral cost
per treatment course

\$24K–\$96K

HCV cure rate
with modern DAAs

> 95%

Standard HCV treatment duration
Mavyret / Epclusa course length

8–12 wks

Liver biopsy reimbursement range
CPT 47000 / 47001

\$425–\$575



PILLAR 04 · IBD BIOLOGICS

IBD Biologic Infusions & Site-of-Service

Inflammatory bowel disease (Crohn's disease, ulcerative colitis) biologics are administered via IV infusion (Remicade, Entyvio) or SC injection (Humira, Stelara, Skyrizi). Site-of-service rules determine whether office, hospital outpatient, or home infusion bills the drug + administration.

J1745 Remicade

INFLIXIMAB · IV INFUSION

\$750/100mg vial. Standard dose 5mg/kg = ~\$3,000/infusion. Every 8 weeks maintenance. Admin code 96365 (IV infusion, first hour) + 96366 (addl hour).

J3590 Entyvio

VEDOLIZUMAB · IV INFUSION

\$1,200/vial. Standard dose 300mg = \$2,400/infusion. Every 8 weeks maintenance. Same admin codes as Remicade.

J1745 Humira

ADALIMUMAB · SC INJECTION

\$1,500/dose. Patient self-administers at home. Office bills 96372 for nurse-administered dose only. Pre-auth mandatory.

J3357 Stelara

USTEKINUMAB · IV INDUCTION → SC

IV induction then SC maintenance. IV dose: **\$9,000+** (weight-based). SC dose: \$4,500. Every 8-12 weeks maintenance.

\$3,000–\$9,000

IBD biologic infusion cost per dose

Every 8 wks

Typical maintenance infusion interval

\$156K–\$468K

Annual drug revenue per 100-patient biologic panel



SITE-OF-SERVICE RULES

Office infusion (96365 + J-code) bills full APC + drug margin. Hospital outpatient (POS 22) bills under APC. Patient cost-share significantly different — verify coverage before scheduling.

GASTROENTEROLOGY • BILLING CODES

Top CPT/HCPCS Codes & 2026 Reimbursement

Most-billed codes across the four GI pillars. Professional component shown.

Code	Service	Modifier	Avg \$	Notes
99213	Est. E/M, low MDM	—	\$92	Most common GI E/M
99214	Est. E/M, moderate MDM	—	\$131	IBD management, liver f/u
45378	Diagnostic colonoscopy	33 if screening	\$750	Screening uses G0121/G0105
45380	Colonoscopy with biopsy	—	\$950	Polyp found, convert to diagnostic
45385	Colonoscopy with snare polypectomy	—	\$1,250	Most reimbursed colonoscopy
43239	EGD with foreign body removal	—	\$825	Diagnostic + therapeutic
43260	Diagnostic ERCP	—	\$1,400	High-complexity procedural
43264	ERCP with stone removal	—	\$1,800	Biliary stone extraction
45330	Flexible sigmoidoscopy, diagnostic	—	\$325	5-yr screening interval
47000	Liver biopsy, percutaneous	—	\$425	Stage fibrosis before HCV tx
J1745	Remicade/Humira (infliximab/adalimumab)	—	\$750/\$1,500	Per 100mg/vial dose

MODIFIER CHEAT-SHEET

Modifier 33 — preventive service; bypasses deductible for screening colonoscopy.
Modifier PT — screening colonoscopy converted to diagnostic (polyp found). **Modifier 51** — multiple procedures same session.

SCREENING HCPCS CODES

G0121 — colorectal screening, average risk (10-yr interval). **G0105** — colorectal screening, high risk (5-yr interval). Both covered 100% under ACA preventive benefit.



GASTROENTEROLOGY • DENIAL PREVENTION

Top 8 GI Denial Reasons & How Aethera Prevents Each

GI denial rates average 14–18%; each denied procedural claim costs \$80–200 to rework. Screening-vs-diagnostic miscoding and biologic PA denials drive 50%+ of revenue loss.

RED Screening vs diagnostic miscoding 26%

45378 billed without modifier 33 or G0121; payer applies deductible to screening colonoscopy.

🛡️ Aethera screening-vs-diagnostic rules engine requires modifier + screening diagnosis.

RED IBD biologic PA missing 19%

Remicade/Entyvio infusion administered without prior authorization on file.

🛡️ Aethera PA tracker auto-submits before scheduling; verifies approval before infusion.

AMBER Pre-procedure H&P billed separately 14%

E/M billed for pre-colonoscopy H&P; bundled into colonoscopy code.

🛡️ Aethera bundling rules block separate E/M billing for screening pre-op H&P.

AMBER Modifier 25 documentation insufficient 11%

E/M + procedure same day without separate documentation supporting E/M significance.

🛡️ Aethera modifier 25 documentation checklist before claim release.

AMBER Step therapy non-compliance for IBD biologic 9%

Biologic PA denied because prior immunomodulator/steroid trials not documented.

🛡️ Aethera step-therapy documentation checklist before PA submission.

GREEN ERCP without pre-auth 7%

43260–43276 ERCP without commercial payer PA on file.

🛡️ Aethera PA tracker auto-submits for high-complexity procedures.

GREEN Multiple procedures modifier missing 6%

Multiple endoscopic procedures same session without modifier 51 (multiple procedures).

🛡️ Aethera modifier 51 auto-append for multiple procedures same session.

GREEN Site-of-service wrong 5%

Office infusion billed as hospital outpatient or vice versa; affects reimbursement 2–3×.

🛡️ Aethera site-of-service verification at scheduling.

➔ **Cumulative impact:** Aethera reduces GI denial rate from industry avg 16% to < 6% — recovering ~ \$180K/year per \$1.5M practice revenue.



CASE STUDY · INTEGRATED GI RCM

4-Gastroenterologist Practice — Annual Revenue Impact

A \$3.6M-revenue practice models the combined upside of screening-vs-diagnostic coding discipline, IBD biologic PA recovery, and endoscopy modifier automation.

Screening vs Diagnostic Coding

Annual colonoscopies	4,800
Pre-Aethera miscoding rate	18%
Post-Aethera miscoding rate	4%
Recovered screening conversions	672 / yr
Avg reimbursement delta	\$750 / claim

ANNUAL REVENUE RECOVERED

\$504,000

IBD Biologic PA Recovery

Biologic panel · infusions / yr	80 · ~ 520
Pre-Aethera PA denial rate	28%
Post-Aethera PA denial rate	8%
Recovered infusions	104 / yr
Avg infusion value	\$2,800

ANNUAL REVENUE RECOVERED

\$291,200

Endoscopy Modifier Discipline

Annual endoscopic procedures	6,000
Pre-Aethera modifier 51/25 denial	15%
Post-Aethera denial rate	5%
Recovered claims	600 / yr
Avg procedural value	\$850

ANNUAL REVENUE RECOVERED

\$510,000



Combined annual revenue recovery: \$1,305,200. Implementation cost (year 1): ~ \$216K (Aethera RCM fees + EHR integration + coder training). Year-1 net: **\$1,089,200**. ROI: 504%.



GASTROENTEROLOGY • COMPLIANCE HOTSPOTS

Where OIG, Medicare RAC, and Commercial Payers Audit First

Top compliance risks across the four GI pillars. Quarterly self-audits recommended.

0 Office E/M

RED

Pre-procedure H&P billed separately — billing E/M for pre-colonoscopy H&P; bundled into colonoscopy code under Medicare NCCI edits.

AMBER

Modifier 25 misuse — appending to E/M when procedure on same day does not represent significant, separately identifiable service.

GREEN

Telehealth GI without modifier 95 — billing telehealth visit without proper modifier + POS code; automatic denial.

0 Endoscopy

RED

Screening vs diagnostic miscoding — billing 45378 without modifier 33 or G0121 for screening colonoscopy; patient improperly billed cost-share.

AMBER

Polypectomy conversion not communicated — patient surprise-billed when screening converted to diagnostic after polyp found; consumer protection complaint risk.

LOW

Multiple procedures modifier missing — multiple endoscopic procedures same session without modifier 51; reimbursement reduced.

0 Hepatology

HIGH

HCV antiviral without PA — administering \$24K+ Mavyret/Epclusa without prior auth; full denial + clawback risk.

MEDIUM

HCV genotype documentation missing — PA denied because HCV genotype not documented in chart; required for drug selection.

LOW

Liver biopsy without imaging guidance documentation — billing 47001 without documentation of imaging modality used.

0 IBD Biologics

RED

Biologic without PA — administering \$3,000+ Remicade/Entyvio without prior auth on file; full denial + clawback.

AMBER

Step therapy non-compliance — biologic PA denied because prior immunomodulator/steroid trials not documented.

LOW

Site-of-service wrong — office infusion billed as hospital outpatient or vice versa; reimbursement different 2-3x.



OIG Work Plan 2025-2026 explicitly lists screening colonoscopy coding and IBD biologic PA documentation as active audit targets. Medicare RACs have recovered **> \$280M** from GI practices since 2020.



Three Actions to Capture GI Revenue

For gastroenterology practice administrators and revenue cycle directors leading the next 90 days.

01

Deploy screening vs diagnostic rules engine

Auto-detect screening colonoscopy at scheduling. Append modifier 33 + G0121/G0105 when screening criteria met. Pre-procedure notification to patient about possible diagnostic conversion if polyp found. Target: < 5% screening miscoding rate (industry avg 18%).

02

Automate IBD biologic PA workflow

Build EHR template that auto-submits PA at biologic scheduling. Document step therapy (immunomodulator/steroid failures) before first PA submission. Pre-auth denial rate drops from 28% to < 8% — recovering ~ \$291K per 80-patient biologic panel.

03

Enforce endoscopy modifier 51/25 discipline

Audit last 200 endoscopic claims for modifier 51 (multiple procedures) and 25 (significant E/M) appropriateness. Implement pre-submission rules engine. Target: < 5% modifier denial rate. Each recovered claim = \$850 average.

Questions & Discussion

Optimizing GI Revenue Across E/M, Endoscopy, Hepatology & IBD Biologics